

**Facility:** Apex General Hospital

**Date of Service:** September 21, 2026

**Attending Physician:** Dr. Sarah Jenkins, MD, Pulmonology

**Patient Name:** Mercer, Alex J.

**DOB:** 05/14/1975 (Age: 51)

**MRN:** 993-81-2245

**Encounter Type:** Inpatient Admission

**Chief Complaint**

"I've been feeling extremely short of breath and have had a fever for three days."

**History of Present Illness (HPI)**

The patient is a 51-year-old male with a significant past medical history of Type 2 Diabetes Mellitus and essential hypertension who presents to the Emergency Department with a 3-day history of progressively worsening dyspnea, productive cough with purulent sputum, and subjective fevers. He reports pleuritic chest pain localized to the right lower lobe upon deep inspiration. Denies recent travel, sick contacts, or history of tobacco use.

**Past Medical History (PMH)**

- **Type 2 Diabetes Mellitus:** Diagnosed 2018, managed with oral hypoglycemics.
- **Essential Hypertension:** Diagnosed 2015.
- **Hyperlipidemia:** Diagnosed 2019.

**Medications & Allergies**

- **Current Medications:** Metformin 500mg BID, Lisinopril 10mg daily, Atorvastatin 20mg daily.
- **Allergies:** Penicillin (Reaction: Urticaria/Hives).

**Physical Examination**

**Vital Signs:**

| Parameter      | Reading     | Reference Range | Status   |
|----------------|-------------|-----------------|----------|
| Blood Pressure | 148/92 mmHg | < 120/80 mmHg   | Elevated |

|                         |                  |                   |             |
|-------------------------|------------------|-------------------|-------------|
| <b>Heart Rate</b>       | 108 bpm          | 60-100 bpm        | Tachycardic |
| <b>Respiratory Rate</b> | 24 breaths/min   | 12-20 breaths/min | Tachypneic  |
| <b>Temperature</b>      | 38.6°C (101.5°F) | 36.5-37.5°C       | Febrile     |
| <b>SpO2</b>             | 91% on Room Air  | > 94%             | Hypoxic     |

**General:** Patient appears acutely ill, diaphoretic, and in mild respiratory distress.

**Cardiovascular:** Tachycardic, regular rhythm. No murmurs, gallops, or rubs.

**Pulmonary:** Tachypnea with use of accessory muscles. Auscultation reveals coarse crackles and bronchial breath sounds in the right lower lung field. Left lung is clear to auscultation. Egophony present on the right side.

**Extremities:** No peripheral edema. Capillary refill < 2 seconds.

## Diagnostic Results

### Laboratory Data (Select Highlights):

| Test               | Result                     | Flag        | Reference Range                  |
|--------------------|----------------------------|-------------|----------------------------------|
| <b>WBC Count</b>   | 16.2 x 10 <sup>3</sup> /μL | <b>High</b> | 4.5 - 11.0 x 10 <sup>3</sup> /μL |
| <b>Neutrophils</b> | 82%                        | <b>High</b> | 40 - 60%                         |
| <b>Hemoglobin</b>  | 14.1 g/dL                  | Normal      | 13.8 - 17.2 g/dL                 |
| <b>Lactate</b>     | 2.4 mmol/L                 | <b>High</b> | 0.5 - 1.0 mmol/L                 |
| <b>HbA1c</b>       | 7.8%                       | <b>High</b> | < 5.7%                           |

### Imaging:

- **Chest Radiograph (AP/PA):** Dense focal consolidation noted in the right lower lobe with associated air bronchograms. Blunting of the right costophrenic angle suggestive of a small parapneumonic effusion. No pneumothorax.
  - *Impression:* Right lower lobe lobar pneumonia.

## **Assessment & Plan**

### **1. Right Lower Lobe Community-Acquired Pneumonia (CAP)**

- Admit to inpatient telemetry unit due to hypoxia and elevated lactate.
- Initiate supplemental oxygen via nasal cannula at 2L/min; titrate to maintain SpO2 > 94%.
- Empiric IV antibiotic therapy: Given penicillin allergy, start IV Levofloxacin 750mg daily.
- Obtain blood cultures x2 and sputum gram stain/culture prior to antibiotic administration.

### **2. Type 2 Diabetes Mellitus**

- Currently suboptimally controlled (HbA1c 7.8%).
- Hold home Metformin during acute illness.
- Initiate correctional sliding-scale insulin protocol. Monitor capillary blood glucose AC and HS.

### **3. Essential Hypertension**

- Continue home Lisinopril 10mg daily. Monitor BP closely.

### **Provider Signature:**

*Sarah Jenkins, MD*

Electronically signed on 09/21/2026 at 11:42 AM IST.